

CANCER SURVIVORSHIP CARE

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NATIONAL CANCER SURVIVORSHIP PROGRAMME

Background

- This programme is for patients who have completed their 5-year cancer surveillance at **any public tertiary oncology department** and are cancer free.
- Wellness focused care: to keep our cancer survivors as well as possible from the diagnosis of cancer until the end of life.
- 2 groups: breast cancer and colorectal cancer patients.

These patients will be discharged from SOC under the cancer survivorship programme to the patient's primary care provider (**enrolled HSG clinic**). In NUP, the patients will be empanelled to teamlet.

Role of NUP Clinician

First step down visit:

1. Review the cancer survivorship care plan (hard copy with patient or soft copy uploaded to Epic → Media tab)
2. Review the patient care coordination notes that is updated by the SOC on cancer surveillance plans
3. Add visit diagnosis of "Carcinoma of breast" or "Carcinoma of colon" and add (+) to Epic problem list

Review following areas of care for cancer survivors yearly

- Look out for symptoms associated with cancer disease recurrence / metastasis
 - **If present, patients should be referred back to the relevant oncology / surgery specialists for further evaluation via survivorship direct access referral pathway (indicate in referral that this patient is under community survivorship follow-up)**
- Be aware of long-term treatment side effects and its management
- **Ensure cancer surveillance is performed, at appropriate intervals and update patient care coordination notes**
- Provide screening and preventive care as per HPB guidelines, including
 - Cardiovascular risk factor screening (DM, Hyperlipidaemia, Hypertension)
 - Age-appropriate cancer screening
 - Bone health
 - Immunisations
- Promote the benefits of healthy living, including diet & exercise, for patients following cancer treatment.

COLORECTAL CANCER SURVIVORSHIP

What to do when you see a colorectal cancer survivor in clinic for their annual follow up?

SmartPhrase for documentation .NUPDRONCOCOLORECTALCASURVIVOR

1. History and physical exam
 - Symptoms that may be associated with disease recurrence / metastasis:
 - Weight loss, abdominal pain, changes in bowel habits from baseline, haematochezia, changes in stool caliber from baseline, symptoms associated with bowel obstruction, jaundice, back/bone pain, persistent cough / dyspnoea, persistent headaches
 - **If symptomatic, perform** exam: In particular, look out for lymphadenopathy (neck, axilla, inguinal), abdominal masses, ascites, hepatomegaly, jaundice, leg swelling
 - **Symptomatic for possible recurrence: Refer back Direct Access to Colorectal Surgery (Indicate: Cancer survivor) if haemodynamically stable, otherwise to ED**
2. Look out for late or long-term side effects
 - E.g. Chronic diarrhoea, bowel / bladder control issues, ostomy issues, neuropathy
 - **Refer back Direct Access to Colorectal Surgery (Indicate: Cancer survivor) for suspected surgery complications, medical oncology for suspected chemotherapy side effects**
3. Is the colonoscopy up-to-date? Check last colonoscopy done and the scope report. Update care coordination notes.
 - Check last colonoscopy report.
 - If last colonoscopy is normal, refer for colonoscopy every 3-5 years until the age of 75, or until life expectancy < 10 years **(Note: Refer to Colorectal Surgery; do not use open access colonoscopy pathway)**
 - If last colonoscopy is abnormal, check specialist's notes / handover for plans on further management.
 - Note: some patients may opt for no further colonoscopy surveillance or other surveillance methods such as CT scan. Check cancer survivorship plan.
4. No need for routine CEA or CT scans
5. Screening and preventive care as per HPB guidelines
 - E.g. CVRF screening, cancer screening (e.g. mammogram or HPV testing), bone health, immunisations

BREAST CANCER SURVIVORSHIP

What to do when you see a breast cancer survivor in clinic for their annual follow up?

SmartPhrase for documentation .NUPDRONCOBREASTCASURVIVOR

1. Annual History
 - Symptoms associated with disease recurrence / metastases: new breast symptom, pathological bone pain, increasing shortness of breath (although a variety of reasons not related to cancer may be the cause), jaundice, headaches with red flags
 - Late or long-term side effects e.g. fatigue, menopausal symptoms, numbness, pain
 - Check on psychosocial health and physical function
2. Annual physical examination: **Breast Exam**, Lymphadenopathy (supraclavicular, axilla). **If symptomatic and as clinically indicated:** Abdominal masses, ascites, hepatomegaly, Jaundice, Pleural effusion
3. Annual mammogram surveillance till 75 years old or life expectancy < 10 years. Check when the patient's last mammogram was. Update care coordination notes.
 - See next page on viewing mammogram result and how to order BSS mammogram
4. **Refer back Direct Access to Breast Surgery (Indicate: Cancer survivor) for recurrence / suspected surgery complications or Medical Oncology for suspected chemotherapy side effects**
5. No need for any regular blood tests or other investigations such as metastatic screen / tumour markers
6. Screening and preventive care as per HPB guidelines
 - E.g. CVRF screening, cancer screening (e.g. FIT / colonoscopy or HPV testing), bone health, immunisations

Breast Cancer Survivorship:

Booking Mammogram under Breast Screen Singapore (BSS)

NUHSD will also help to book for patients for the following year. Patient to self-book annual BSS mammogram 2-3 months before doctor appointment if not yet booked.

1. NUHS Diagnostics hotline at 6370 6556,
2. email NUHSD at nuhsd_contact@nuhs.edu.sg
3. Approach NUHS Diagnostics counter staff at NUP
4. HealthHub App

Patient will receive BSS results letter within 3-4 weeks, up to 6 weeks.

There is no medical report or X-ray films provided as the results are recorded in the BSS system (available on NEHR). If results are abnormal, the notification result letters with instructions to book an appointment at NUH assessment centre will be sent to patient. Further evaluations such as additional mammograms, ultrasounds, tomosynthesis, including biopsies, may be performed on the same assessment day.

Viewing last mammogram report

- Mammogram reports under BSS can be found in NEHR under “Screening/Indicators → Medical Screening”
- Mammograms done in hospitals can be found in NEHR under “Investigations → Radiology/Nuclear med”

If latest BSS mammogram is abnormal, ensure patient has been recalled by BSS for further evaluation. If not, to refer to NUH Breast Clinic direct access.

CONTACT RESOURCES AT NUHS

Cancer Appointment-Related Enquiries:

Phone: (+65) 6773 7888 | **Email:** CancerApptLine@nuhs.edu.sg

Questions for our Oncology Nurse:

Phone: (+65) 9722 0569 | **Email:** CancerLineNurse@nuhs.edu.sg

Questions for our Stoma Care Nurse:

Phone: (+65) 8781 2378

CANCER REHABILITATION AND COMMUNITY SERVICES

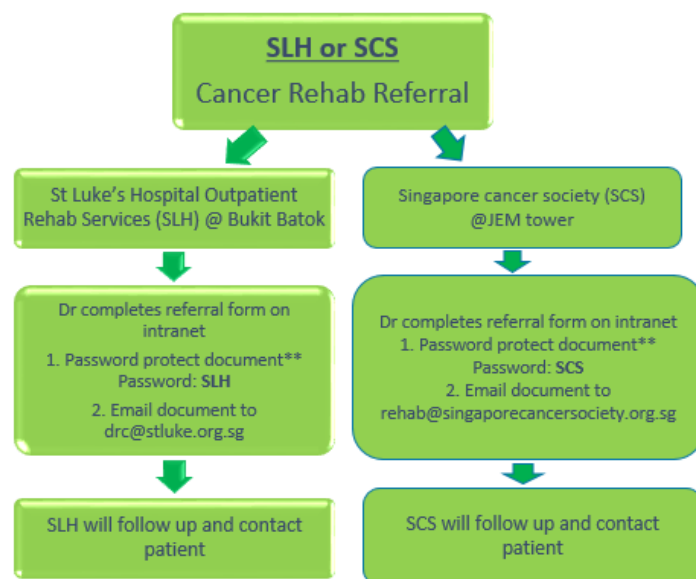
Available at:

- NUH / NTFGH Rehabilitation Medicine (Referral via Epic)
- [Singapore Cancer Society Rehabilitation Centre or Cancer 365](#) (refer to MSW)



Access our Resource Hub
<https://365cps.org.sg/resourcehub/>

- [St. Luke's Hospital Outpatient Rehabilitation Service/Epic referral](#) (Applicable to BBK only)



CANCER REHAB SERVICES

Discipline	Services	NUH	NTFGH	SCS	SLH
Physiotherapy	Musculoskeletal management (head & neck, shoulder dysfunction, pain, deconditioning, radiation fibrosis, post-op scar tissue, ortho issues)	X	X	X	
	Neurological issues (balance, coordination, fall prevention)	X	X	X	X
	Cardiopulmonary (dyspnea, secretions management)	X	X	X	
	Lymphedema management (upper limb)	X		X	
	Lymphedema management (lower limb)	X		X	X
	Caregiver training	X	X	X	
	Patient education	X	X	X	X
Occupational therapy	Lymphedema management (upper limb)	X	X	X	
	Lymphedema management (lower limb)	X	X	X	X
	Cancer related fatigue	X		X	
	Chemotherapy induced peripheral neuropathy (CIPN)	X		X	
	Return to work	X	X	X	X
	Activity of daily activities / home engagement	X	X	X	X
	Community integration	X	X		X
	Arthralgia such as De Quervain, trigger fingers	X	X	X	
	Caregiver training	X	X	X	X
	Patient education	X	X	X	
Speech therapy	Swallowing dysfunction	X	X	X	X
	Speech and communication dysfunction	X	X	X	X
	Caregiver training	X	X	X	
	Patient education	X	X	X	
Dietetics	Weight management	X	X	X	
	Fatigue management			X	
	Malnutrition	X	X	X	
	Cooking demonstration workshops	X	X	X	
	Caregiver training	X	X	X	
	Patient education	X	X	X	
Exercise physiology	Individual and supervised group based exercise			X	
	Transition to Active SG gyms			X	
	Patient education			X	
Psychosocial	Emotional capability (dealing with self, family, colleagues, society)	X		X	
	Financial capability	X		X	
	Vocational capability	X		X	
	Sexuality challenges	X		X	